

**FOR YOUR DEVELOPER**

Every feature, and what we are allowed to say about it

This document is for your developer.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

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VERSION

1.0 · Confidential draft

This document is for your developer.

It maps every feature the founder has described to a version number, a legal position and the proof needed before anyone claims it works.

In one minute

- Most of the founder's ideas fit version 1 as coordination and education, with no clinical judgement.
- Three ideas carry medical-device risk: triage sorting, blood-sugar interpretation, and any wearable.
- Voice tracing of baby movements is a research idea. It must never appear in sales material as a feature.
- Replacing a hospital system and joining one are two settings of the same product, not two products.
- The old prototype already suggests clinical actions. That must be removed before anyone sees it again.

What to do

Action	Who	By when
Get a clinician to review the intended-use statement below	Founder	Day 14
Make the four prototype changes at the end of this document	Developer	Day 14
Scope a regulatory opinion on triage and blood sugar	Founder and counsel	Day 21
Remove voice movement tracing from every deck and page	Founder	Day 7
Build version 1 as Core plus Engage plus voice storage only	Developer	Build start

The feature map

Version 1 is what we build now. Version 2 needs a regulatory opinion first. Version 3 is research, not product.

What the founder described	What we actually build	Version	Legal position	Proof needed before we claim it	Price code
E-care patient database, the "maternity watch" record	Core register: mother and newborn, timeline, notes, exports, fields the provider can configure	v1	Record support, no clinical judgement. Never the legal record where one already exists	DPIA, security certification, audit log tests	ML-CORE
Given at the week-16 midwife appointment, returned after postnatal discharge	Enrolment and discharge workflow: consent, onboarding, device issue and return log, offboarding, retention rules	v1 software, device later	Software: nothing beyond data protection. Device: see the wearable row	Onboarding time and completion rates in the pilot	ML-CORE, plus ML-WATCH
Midwife can explain things better	Education library signed off by clinicians, delivered in the app and by SMS or WhatsApp, more languages later	v1	Educational content under a sign-off process	Comprehension and engagement measures	ML-ENGAGE

What the founder described	What we actually build	Version	Legal position	Proof needed before we claim it	Price code
Shorten the appointment	Pre-appointment check-in and a symptom summary for the clinician; structured handover	v1	A summary with no judgement. The clinician reads the raw entries	A time-and-motion study in the pilot. Do not claim minutes saved before they are measured	ML-CORE
Maternity triage, maternity A&E	Triage intake: structured capture of what she is presenting with, contact and outcome logging, and sorting rules owned by the site's own clinical governance	v2, test first	Likely clinical decision support if Vytalix ships the rules. Safe position: ship the rule editor with no default rules. Get an MHRA classification opinion	Regulatory opinion, then a service evaluation at one site	ML-TRIAGE
Daily questionnaire, like a period app	Daily check-in with configurable questions, streaks and reminders. Answers visible to the care team	v1	No clinical judgement. Every concerning answer goes to a human within a set deadline	Engagement rates; an audit of how escalations were handled	ML-ENGAGE
Quizzes and a section for partners	Partner account linked with her consent; education quizzes; prompts such as appointment companion and warning signs	v1	Consent and safeguarding. She controls partner access and can revoke it silently, for domestic-abuse safety	Safeguarding review; user testing with charities	ML-ENGAGE
Log symptoms such as itching hands and feet, or vomiting	Symptom log with plain-language prompts. Information only: "itching of hands and feet in pregnancy should be reported to your midwife today", with one-tap contact. The software never produces a diagnosis label	v1	No clinical judgement as long as the software neither scores nor interprets. Wording reviewed by clinicians	Content sign-off; a hazard log entry for each symptom	ML-ENGAGE
Women with diabetes enter blood-sugar readings	Self-reported glucose diary with targets set by the clinician, a trend view, and a threshold notification to the care team	v2, deferred	Threshold alerting on physiological data moves towards being a medical device. Get an opinion before building	Regulatory opinion; design work with clinicians	ML-GLUCOSE
Voice tracing of babies' movements	Not a feature. A research question: capturing movement patterns needs a validated sensor, ethics approval, a clinical partner and a device pathway	v3 research	A medical device by its purpose, if it informs care	Ethics approval, a protocol, a data partnership, a validation study	none
Voice diaries	Storage and playback attached to the timeline. Transcription and translation later	v1	Storage only. No analysis	Consent flows; retention policy	ML-VOICE

What the founder described	What we actually build	Version	Legal position	Proof needed before we claim it	Price code
Maternity watch, the wearable	Joining a certified wearable to the record for prompts and, where the device is certified for it, readings shown without interpretation	v2 or v3, partner device	The device carries its own UKCA or CE marking. We display, we do not interpret	Supplier checks; the device's regulatory status; DPIA update	ML-WATCH
Replacing hospital systems	Deployment mode A: MaternaLink Core as the programme's working record where none exists	v1	Needs a data-protection and clinical-safety case for record keeping. Still not a certified hospital record system	Clinical safety case	ML-CORE plus implementation
Joining hospital systems	Deployment mode B: interfaces to BadgerNet, K2, Euroking, DHIS2 or the state health information system, and single sign-on	v1 to v2	Standard integration work under vendor agreements	Interface tests; vendor sign-off	ML-INTEGRATE

The intended-use statement

This is the draft wording. A clinician and a lawyer must review it before it appears anywhere.

"MaternaLink is a care-coordination and communication platform that helps maternity programmes enrol women, keep in contact between appointments, record self-reported information and clinician notes, and route reported concerns to the responsible clinician. It does not diagnose, predict, triage or recommend treatment. Any prioritisation rules are configured, owned and reviewed by the deploying organisation's clinical governance."

Every screen, every deck and every proposal must stay inside that sentence. If a feature cannot be described within it, the feature needs a regulatory opinion first.

The four changes the prototype needs before anyone reuses it

The prototype running on Vercel already produces a risk score and suggests clinical actions. That is the single biggest gap between what exists and what we are allowed to claim.

#	Change	Why
1	Remove or relabel the "suggested actions" output so no clinical recommendation is produced by default. Replace it with "Programme escalation rule: [name] — contact the responsible clinician"	The software must not recommend treatment
2	Remove identifiable photographs and real-looking test identities. Use synthetic data	Nobody should be able to mistake demo data for a real person
3	Put the public URL behind a login, and confirm no real patient data was ever entered	An open URL with patient-like data is a data-protection incident waiting to happen
4	Rename it to MaternaLink, add the Vytalix mark, and match the wording in this document	The old naming and copy claim more than we can support

What to build first, and what to hold

Position	Detail
Build now	Core, Engage and voice storage. These are the version 1 product
Hold	Triage and glucose, until a regulatory opinion is written down
Partner, do not build	The wearable. Someone else carries the device certification
Never claim	Voice movement tracing as a feature; any reduction in minutes, risk or harm that has not been measured

Risks to watch: features creeping back towards decision support; partner access failing a safeguarding test; over-promising the watch to an investor.

Words explained

Word	What it means
Medical device	Software that diagnoses, predicts, monitors or guides treatment. It needs approval and a quality system before sale
Clinical decision support	Software that tells a clinician what to do or how urgent something is. That is a device
MHRA	The UK regulator that decides whether software counts as a medical device
UKCA or CE marking	The mark showing a device has passed the approval route. MaternaLink has neither and does not need one while it stays non-diagnostic
DPIA	The written check that a product handles personal data lawfully
Clinical safety case	The written argument, signed by a named safety officer, that the software is safe to use in care
Service evaluation	Watching a service that is already running. It is not research and it proves nothing about effectiveness
Synthetic data	Invented records that look real but belong to nobody
PROPOSED	This is a plan we have written down, not a decision anyone has approved